Paracetamol

Mechanism of action

- Paracetamol inhibits the production of prostaglandins resulting in a reduction in pain and fever.

Authorised practice levels

- EMTs, Paramedics, ICPs and CCPs.
- First Responders: refer to 'pain relief' in the FR Field Guide

Indications

- ✓ Mild or moderate pain.
- ✓ Moderate to severe pain, in addition to other medicines.

Contraindications

- ✕ Known severe allergy.

Cautions

- ⚠ The patient has taken paracetamol within the last four hours. Paracetamol is contained in many products such as cold and flu tablets/drinks, cough mixtures, combination pain relievers and migraine tablets. Additional paracetamol may be administered provided it is clear that the total dose taken within a four hour period does not exceed the dose described within the CPGs. Withhold paracetamol if there is any doubt.
- ⚠ Abdominal pain, particularly if the patient is very unwell or vomiting. Clinical judgement is required, but if the patient is very unwell or vomiting, the possibility of significant intra-abdominal pathology exists and oral medicines should usually be withheld.
- ⚠ Known severe liver disease. Liver disease must be severely impaired before paracetamol clearance is altered, but the balance of risk is such that paracetamol should usually be withheld in this setting.

Use in pregnancy or when breastfeeding

- Safe and may be administered if indicated.

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Dosage

- 1.5 g PO for an adult weighing greater than 80 kg.
- 1 g PO for an adult weighing 80 kg or less.
- See the paediatric drug dose tables for a child.
- **First Responders:** refer to '**pain relief**' in the FR Field Guide and administer the dose listed there.

Administration

- Administer PO.
- Children unable to swallow tablets may be administered paracetamol syrup, or tablets that have been crushed and placed in a soft food such as jam or honey.

Common adverse effects

- None.

Usual onset of effect

- 30-60 minutes.

Usual duration of effect

- 4-6 hours.

Usual preparation

- 500 mg tablets.
- Syrup containing 50 mg/ml.

Pharmacokinetics

- Paracetamol is metabolised in the liver.

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- If liver impairment is severe, paracetamol clearance will be significantly delayed, but this does not affect the initial (loading) dose.

Common interactions

- None.

Additional information

- Paracetamol should be administered in addition to other medicines for severe pain, particularly if the transport time is long. This is not a priority but will reduce the need for subsequent analgesia and improve the quality of pain relief.
- The described doses of paracetamol are higher than those usually recommended in many references. However, the described doses are safe provided they are not administered repeatedly.
- Paracetamol is not indicated for the treatment of fever because fever may confer some benefit if the patient has an infection. However, paracetamol may be administered if the patient has a temperature greater than 39°C and the fever is causing discomfort.
- Paracetamol is not indicated for pain associated with myocardial ischaemia.
- All personnel (including those without ATP) may give paracetamol to a patient for self-administration, provided the package instructions are followed.
- A patient may be administered paracetamol for pain associated with a minor condition and be given a recommendation that immediate referral to a medical facility is not required.

More Information

Version history

22 November 2022 1.0.1

Update: Aligned with analgesia CPG.